

Endoscopic Urethroplasty (TUITMR)

Patient Instructions · A no-incision repair of a scar at the bladder neck

WARWIKI

This is a repair of a tight scar at the bladder neck, or where the bladder was reconnected after prostate surgery, done entirely through a scope — with no outside incision. Your surgeon opens the scar and then stitches healthy bladder lining across it, so the area heals open instead of scarring shut again.

About This Procedure

After prostate surgery or radiation, a tight scar can form at the **bladder neck**, or at the join between the bladder and urethra, and block the urine stream. Simply cutting the scar often lets it close again, because it heals over raw scar tissue.

This technique — **TUITMR** (transurethral incision with transverse mucosal realignment) — solves that problem:

- The surgeon **cuts the scar** through a scope to open the channel.
- Then, using a special **suturing tool**, healthy bladder lining is brought across and stitched over the cut — like a flap repair, but through the urethra.

Early results are encouraging: about **9 out of 10** stay open after one procedure, and nearly all after a second — with very little risk of new leakage in the first reported group.

Is It Safe?

It is done entirely through the urethra, avoiding the larger robotic or open operations used for this problem. Early evidence is promising, including a low risk of new urine leakage, which can complicate other repairs here. Risks include bleeding, a urine infection, and the narrowing returning. It is a newer, specialized technique.

LEARN THE TERMS

Bladder neck

The outlet where the bladder funnels into the urethra.

Bladder neck contracture

A tight scar at the bladder neck that blocks the urine stream.

Anastomotic stenosis

A scar narrowing where the bladder was reconnected after prostate surgery.

Urethra

The tube that carries urine out of the body.

Mucosa (lining)

The healthy inner lining advanced across the scar to heal it open.

Cystoscope

The thin camera the surgeon works through.

Incontinence

Leaking urine — a risk this technique aims to keep low.

Catheter

A soft tube left in the urethra for a short time while it heals.

WILL IT HURT? The procedure is done under anesthesia, so you feel nothing during it. Afterward, expect mild burning and some blood-tinged urine for a few days. A catheter is left in place for a short time. Most people go home the same day or after a short stay.

How to Get Ready (Before the Procedure)

- Done under **general anesthesia** — follow all surgery instructions (fasting, and which medicines to hold, including blood thinners).
- A **urine test** checks for infection — an infection must be treated first.
- Plan for a **catheter for a short time**, and arrange a ride home.

Tell your team ahead of time if you:

- Take a **blood thinner**, or have a current urine infection
- Have had **prostate surgery, radiation**, or prior treatments to this area

What Happens During the Procedure

- 1 You are asleep under anesthesia and antibiotics are given.
- 2 A scope is passed to the scar, and a guidewire is placed for safety.
- 3 The scar is widened and cut to open the channel.
- 4 Using a suturing tool passed through the urethra, healthy bladder lining is advanced and stitched across the cut, and a catheter is left in place.

After the Procedure

- You go home the **same day or after a short stay**, with a catheter for a short time.
- Expect **mild burning and pink urine** for a few days. Drinking extra water helps.
- **Avoid heavy lifting and straining** for a few weeks.
- A follow-up scope check (around **4 months**) confirms the area stays open.

Call your care team or seek care if you have:

- A fever or chills
- You **cannot urinate**, especially after the catheter is out
- New heavy **leakage** of urine
- Heavy bleeding, blood clots, or worsening pain

THREE THINGS TO REMEMBER

1. This is a no-incision repair of a scar at the bladder neck — the scar is opened and covered with healthy lining so it heals open, with a low risk of new leakage.
2. To prepare: follow your fasting and medicine instructions, treat any infection first, plan for a catheter for a short time, and arrange a ride.
3. Mild burning and a little pink urine for a few days is normal, and a follow-up scope confirms success. Call right away for fever, inability to urinate, new heavy leakage, or heavy bleeding.